



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : هناء عبدالمنعم سليمان عبده

المحافظة : الجيزة الحي / المدينة : بولاق الدكرور

رقم البطاقة : 25712300100401 رقم التليفون : 01110381972

العنوان : ش محمد نافع

العمر (تاريخ الميلاد) : 30-12-1957 المهنة : ربه منزل تاريخ تسجيل الإستمارة : 03-10-2026

الإقرار

أقر أنا الموقع أدناه أنني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات أو أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي (.....) التوقيع: (.....)

المقرر بما فيه (.....) التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] قسم العلاج : الاورام

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني الوظيفة : التوقيع :
الاسم : د/ندى حسن الوظيفة : التوقيع :
الاسم : د/نرمين رمضان الوظيفة : التوقيع :

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة رئيس اللجنة ومدير المستشفى.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

..... د/..... د/..... د/..... د/.....

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى : قسم :
جهة العلاج : نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: هناء عبدالمنعم سليمان عيده National ID: 25712300100401	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حمید بالثدي و تحتاج الي Supportive and palliative Treatment
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

MRN: 2021170692 D.O.B: 30/12/1957
 Patient Name: HANAA ABD ELMANAM SOLIMAN ABDO Sex: Female
 IC No.: 25712300100401
 Location: External Clinic

SURGICAL PATHOLOGY

Specimen examination - (1 Site)

Specimen Id: 2608030023 Collection Date: 03/08/2026 09:07 AM
 Accession No: SUR-26-2723 Received Date: 03/08/2026 09:07 AM
 Sample Type: Tissue Doctor: Eman Sobhy Adrawos Mahgoub

Clinical Information

Excision biopsy from neck of femur after total hip orthoplasty.
 Known case of Lt. breast Invasive duct carcinoma, grade 2 post NACTH (pathology no. 7623-22).

Gross Picture

Multiple hard and soft tissue fragments measuring collectively 6x5x3 cm) totally embedded.

Microscopic Picture

A case of bone fragments infiltrated by groups pf malignant epithelial cells. Picture is consistent with metastatic adenocarcinoma.
 Gata3 showed negative staining

Diagnostic Summary

Tumour type:
 Metastatic adenocarcinoma.

Specimen type:
 excision biopsy

Diagnosis

Neck of femur, Excision biopsy:

Positive for metastatic adenocarcinoma.
 Gata3 negative immunoreactivity could be attributed to preanalytical decalcification solution, however Breast origin cannot be excluded.

Comments

Gata3 is ordered on block A+H

Signature

Heba A Abdelbaky (MD)

Co-signed by:

MRN: 2021170692
Patient Name: HANAA ABD ELMANAM SOLIMAN ABDO
IC No.: 25712300100401
Location: External Clinic
D.O.B: 30/12/1957
Sex: Female

IMMUNOPATHOLOGY

Her2 Receptors

Specimen Id 2110230338
Accession No IMM21-7132
Sample Type Paraffin Block
Collection Date 23/10/2021 11:27 AM
Received Date 23/10/2021 11:27 AM
Doctor Abd ALLAH Mohamed Gamil

Immunohistochemical Report

pathology number
8482/21

Clinical Data

It. breast cancer

Gross Picture :

Referred block submitted for HER 2 immunostaining.

System Used :

Ventana BenchMark ULTRA autostainer.

Antibody:

Ventana anti-HER-2 [4B5] rabbit monoclonal antibody.

Scoring system :

Reference: ASCO-CAP HER2 Test Guideline Recommendations, 2018. IHC 0 :as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within $\leq 10\%$ of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within $>10\%$ of the invasive tumor cells. IHC 2+: The revised definition of IHC 2+ (equivocal) is weak to moderate complete membrane staining observed in $>10\%$ of tumor cells.. IHC 3+: based on circumferential membrane staining that is complete, intense in $>10\%$ of tumor cells.

(Laterality) breast Cancer:

left

Her2 Result

Negative [score 0]

Signature

GHADA MOHAMED MD PHD.

Validated on 30/10/2021 07:30 PM

Print Date:

26/09/2026 12:43 PM

Page Number: 1 of 2



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مركز بيية الهرم
4 شارع علوية أول شارع الهرم بجوار داري , الجزيرة



مستشفى بيية الشيخ زايد
14 الدى الأول - الشيخ زايد أمام كلية الهندسة

MRN: 2021170692
Patient Name: HANAA ABD ELMANAM SOLIMAN ABDO
IC No.: 25712300100401
Location: External Clinic

D.O.B: 30/12/1957
Sex: Female

IMMUNOPATHOLOGY

Hormone receptors

Collection Date 23/10/2021 11:27 AM
Received Date 23/10/2021 11:27 AM
Doctor Abd ALLAH Mohamed Gamil

Specimen Id 2110230337
Accession No IMM21-7131
Sample Type Paraffin Block

Immunohistochemical Report

pathology number 8482/21

Clinical Data
Lt. breast cancer

Gross Picture:
Referred block submitted for ER/PR immunostaining.

System Used:
Ventana BenchMark ULTRA autostainer.

Antibody:
Ventana anti-PR[1E2] Rabbit monoclonal antibody, Ventana anti-ER[SP1] Rabbit monoclonal antibody.

Scoring system :
According to the ASCO/CAP guideline update, 2019 : Samples with 1% to 100% of tumor nuclei positive for ER or progesterone receptor (PgR) are immunoreactive, the tumor nuclei positive for ER (not PgR), 1% to 10% of tumor cell nuclei are immunoreactive for ER or positive. If ER (not PgR), 1% to 10% of tumor cell nuclei are immunoreactive. Allred score: sample are reported as ER Low Positive. A sample is considered negative for ER or PgR if < 1% or 0% of tumor cell nuclei are immunoreactive. Allred score: 1 and 10%; 3, proportion of cells staining (0, no staining; 1, <1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate and 3, strong). Total score from 0 to 8. Comment: - There are much more limited data on the 2% to 3% of cancers that are low ER expressers (most often defined as 1%-10% ER-positive cells by IHC) and their potential benefit from endocrine therapy. - PR positive in 1-20% of tumor cells is considered as Low positive.

(Laterality) breast Cancer:

left
Reaction in 100% of tumor cells [8/8].

Diagnosis :
ER: Positive, strong nuclear reaction in 100% of tumor cells [8/8].

stological grade (1-3):

eritumoural lymphovascular invasion

bsent

liagnosis

t. breast retroareolar mass, Tru-cut biopsy:
[invasive duct carcinoma, grade II.

Comments

ER, PR and Her2 are ordered.

Biopsy reporting category

B5b= Invasive cancer

Signature

Basant Abd Elmoneam Ahmed

Co-signed by:

[“GHADA MDHAMED MD PHD.”]

Reported and electronically endorsed by

Ghada Mohamed Abdulsalam Ahmed

Pathology lab director:
Dr. Hazem Abdelazim, MD, IFCAP

Page Number: 2 of 2

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Print Date:

26/09/2026 12:42 PM



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مركز بيهة اليوم
4 شارع علوية أول شارع اليوم بجوار داري , الجزيرة



مستشفى بيهة الشيخ زايد
٤٠ الحي الأول - الشيخ زايد، كنفكة، مدينة خليفة، أبو ظبي

MRN:

Patient Name:

IC No.:

Location:

2021170692

HANAA ABD ELMANAM SOLIMAN ABDO

25712300100401

Surgical Oncology

30/12/1957

D.O.B:

Female

Sex:

SURGICAL PATHOLOGY

Biopsy Small - (1 Site)

Specimen Id

Accession No

Sample Type

2110200345

SUR21-8482

Tissue

20/10/2021 02:37 PM

Collection Date

20/10/2021 02:38 PM

Received Date

Doctor

ahmed mahmoud samy Abd Almonem

Clinical Information

Lt. breast retro-areolar mass, BIRADS 5.

Gross Picture

Multiple cores 0.8-1.2 cm.

Microscopic Picture

A case of invasive duct carcinoma, grade II with focal lobular differentiation. There is DCIS component , 3%. Stroma is desmoplastic with mild inflammatory cell reaction.

Diagnostic Summary

Tumour type:

Invasive duct carcinoma, grade II.

Laterality

Left

Specimen type:

Tru-cut biopsy.

Location

Retroareolar

Histological grade

Mitosis score (1-3):

2

Tubular formation score (1-3):

3

Nuclear grade (1-3):

2

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26/09/2026 12:42 PM

Page Number: 1 c



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مركز بهية الهرم
4 شارع علوية أول شارع الهرم بجوار داري , الجزيرةمستشفى بهية الشيخ زايد
٤. الحي الأول - الشيخ زايد أمام مكتبة المكتبة

[DOCTYPE:PATHOLOGY REPORT]

Medical Reports

Patient Information

MRN	2021170692	Name	HANAA ABD ELMANAM SOLIMAN ABDO
ID Type	الرقم القومي	ID Number	25712300100401
Gender	FEMALE	Nationality	EGYPTIAN
Written Date	28/09/2026 10:37 am	Visit Date	17/09/2026
Diagnosis	[P][N63]		Unspecified lump in breast

Report and Recommendation

68-year-old woman with left breast grade II invasive ductal carcinoma, initially ER 100% (8/8), PR 90% (8/8) and HER2 IHC 0. She received one cycle of neoadjuvant EC, stopped after persistent hypertension, then started aromatase inhibitor in January 2022 before left modified radical mastectomy. Pathology showed grade II carcinoma with a skin tumor nodule, ypT4N2a, and 4/15 positive nodes. She continued adjuvant letrozole. In June 2026, CT and bone scan showed left femoral-neck and sacral lesions described as metastatic, with a pathological left femoral-neck fracture and no definite pulmonary or hepatic metastases. She underwent left femoral-neck replacement; excision biopsy showed metastatic adenocarcinoma, with breast origin not excluded despite negative GATA3 staining. Fulvestrant was started in September 2026; the plan also included a CDK4/6 inhibitor, zoledronic acid every three months and palliative radiotherapy. September imaging showed dislocation of the left hip prosthesis and a persistent ununited fracture, prompting orthopedic consultation before radiotherapy.

Written By

عمر

Approved By

Hanan Ramadan Nasar Mohamed

Medical Director

د. هاني

بـيـة
عيادات طب الأورام
Oncology Clinics

بـيـة
الإدارة الطبية
Medical Department

Reported By, Dr. Aalaa Sobhy, MD

01-Sep-2025 02:48 PM

Follow up after one year

• Benign findings of the right breast (BIRADS 2).
• Post left MRM showing no evidence of recurrence.

OPINION:

- Nonspecific axillary lymph nodes.
- Normal nipple areola complex.
- Normal skin thickness and contour.
- lesions.
- This was confirmed by Complementary Ultrasound which showed no solid or cystic mass
- No spiculated mass lesions or grouped microcalcifications.
- Benign vascular calcifications
- Scattered fibroglandular densities (ACR B).

Right Breast:

- No regional lymphadenopathy.
- No operative bed mass or collection.

Left MRM OPERATIVE BED:

REPORT:

- Complementary ultrasound
- CC and MLO view.

TECHNIQUE:

- Follow-up after left mastectomy

INDICATION:

ULTRASOUND

RIGHT SIDED FULL FIELD DIGITAL MAMMOGRAPHY AND COMPLEMENTARY

PATIENT NAME: HANAA ABD ELMANAM SOLIMAN ABDO	DATE : 01-Sep-2025 11:45 AM
GENDER: Female	
PATIENT ID: 2021170692	ORDER: 3295562,3295563
AGE: 67 y	

PATIENT NAME: HANAA ABD ELMANAM SOLIMAN ABDO
GENDER: Female
DATE : 15-Mar-2025 10:07 AM

PATIENT ID: 2021170692
AGE: 67 y
ORDER: 3138927

BONE MINERAL DENSITY

SITE	Region	BMineral Density (BMD)	T- score
Forearm	Distal	0.628	-1.1
Femur	Neck	0.705	-1.3
Spine	Average	0.926	-1.1
World Health Organization (WHO)		Criteria For osteoporosis	
Diagnostic Category		T-Score	
Normal		T-Score from +1 to -1	
Osteopenia		T-Score from - 1 to - 2.5	
Osteoporosis		T-Score from - -2.5	
Established osteoporosis		T-Sore < -2.5 (with fractures)	

Comment:

Measured BMD values are indicative of:

- **Generalized Osteopenia**

Reported By,

Dr. Ahmed Rozeka, MSc

16-Mar-2025 06:00 PM



PATIENT NAME: HANA ABD ELMANAM SOLIMAN ABDO

GENDER: Female

DATE :

PATIENT ID: 2021170692

AGE: 66 y

ORDER: 3135640

TRANSTHORACIC ECHOCARDIOGRAPHY

M.M. MEASUREMENTS

LVEDd: 5.5 Cm LA : 3.4 Cm

LVEDS: 3.5 Cm Ao : 3.0 Cm

IVS : 1.1 Cm RV : 2.2 Cm

PWT : 1.0 Cm

EF % : 65 % FS % : 36 %

2 D COMMENT:

- 1- Normal LV internal dimensions with good overall systolic function EF 65 %
- 2- Diastolic dysfunction grade 1 (impaired relaxation pattern).
- 3- No gross resting segmental wall motion abnormalities (RSWMA).
- 4- mild to moderate MR.
- 5- The aortic valve appears sclerotic with max SPG 22 mmHg.
- 6- Normal Rt. Sided dimensions with mild TR, EPASP 40 mmHg.
- 7- No intra-cardiac masses or thrombi
- 8- Normal pericardium, no pericardial effusion

CONCLUSIONS

- Diastolic dysfunction grade 1, mild to moderate MR .
- Mild pulmonary hypertension.

Reported By,

Dr. Medhat Elsayed

25-Nov-2024 05:12 PM

PATIENT: HANA ABD ELMANAM SOLIMAN ABDO, 2021170692, Female, 66 y

Report version 1, 25-Nov-2024 05:12 PM

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مركز بهية التوج
9 شارع حلوة أول شارع الورع بعباد داري، الجيزةمستشفى بهية الشيخ زايد
8 على النيل - الشيخ زايد بعباد كلية الهندسة

PATIENT NAME: HANA ABD ELMANAM SOLIMAN ABDO	PATIENT ID #: 2021170692
GENDER: Female	ORDER #: 2339580,2339581
AGE: 65 y	DATE OF SERVICE: 02-Sep-2023 02:15 PM

Report: - all studies

**FULL FIELD DIGITAL MAMMOGRAPHY AND COMPLEMENTARY
ULTRASOUND**

INDICATION:

- Screening.
- Left MRM.

TECHNIQUE:

- CC and MLO view.
- Complementary ultrasound

REPORT:

- Scattered fibroglandular densities, of the right breast (ACR b).
- No spiculated mass lesions or grouped microcalcifications.
- This was confirmed by Complementary Ultrasound which also showed no right solid or cystic mass lesions.
- Normal skin thickness and contour.
- Normal nipple areola complexes.
- Free left operative bed with no evidence of recurrent masses.
- no supra or infra-clavicular pathological lymph node .
- Nonspecific right axillary lymph nodes.

OPINION:

- Left MRM with no evidence of recurrence
- no right breast suspicious lesions
- (BIRADS2).

Recommendation - all studies

Follow up after one year

Reported By,

Dr. Mennat Allah Samir, MSc

02-Sep-2023 06:04 PM

PATIENT NAME: HANA ABD ELMANAM SOLIMAN	PATIENT ID #: 2021170692
ABDO	ORDER #: 1751816,1751819
GENDER: Female	DATE OF SERVICE: 14-Aug-2022 10:08 AM
AGE: 64 y	

Report: - all studies

**POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST,
ABDOMEN AND PELVIS**

As compared to previous study dated 02-11-2021.

1-CHEST:

- Reduced size of the previous left breast retro-areolar irregular spiculated mass lesion reaching and retracting the skin and nipple measuring 33x28 mm as compared to 52 X 39 mm.
- Reduced size of the previous left axillary level I and II enlarged lymph nodes the largest measures 21x8 mm as compared to 34 X 21 mm.
- No sizable pulmonary nodules the largest is seen at lateral segment of middle lung lobe measuring 5 mm.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- No gross cardiac abnormalities.
- Right thyroid gland hypodense focal lesion measuring 15 X 12 mm. calcified small left thyroid nodule.

2- ABDOMEN AND PELVIS

- The liver is enlarged with homogenous CT texture. Few scattered tiny calcified foci mostly old granulomatous.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with few scattered tiny calcified focal lesions as well as peripheral subcapsular rim calcifications mostly representing post granulomatous sequel.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Left adrenal soft tissue lesion measuring 18 X 11 mm.

- Normal appearance of the pancreas, aorta, I.V.C and right suprarenal.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- The scanned skeleton shows few right iliac, left femoral and humeral heads sclerotic foci mostly bone islands.
- Left femoral head subcapsular sclerotic bone focus is currently seen for functional assessment/close follow-up.

Opinion:

- *Follow up status revealed regressive course of the left breast malignant looking mass lesion as well as metastatic ipsilateral axillary lymph nodes.*
- *No definite evidence of pulmonary or hepatic metastatic deposits.*

Reported By,

Dr. Mahmoud Saleh, MD
Dr. Reham Anwar, MSc, FEBR

14-Aug-2022 05:07 PM

قسم الأشعة التشخيصية

بهدية

مركز بهدية للاكتشاف المبكر
ومعالجة سرطان الثدي

PATIENT NAME: HANA ABD ELMANAM SOLIMAN ABDO	PATIENT ID #: 2021170692
GENDER: Female	ORDER #: 1279953
AGE: 63 y	DATE OF SERVICE: 20-Oct-2021 01:46 PM

Report:

ULTRASOUND GUIDED BIOPSY

BIOPSY

Under aseptic conditions 5 core biopsies form left breast retroareolar locally suspicious lesion (BIRADS 5) for histopathological confirmation, ER,PR and HER2 .

No Complications
Procedure well tolerated

Reported By,

Dr. Eman Hosny, MSc

20-Oct-2021 02:01 PM



جمهورية مصر العربية
بطاقة تعريف الشخصية

هنا
عبد المنعم سليمان عبده
ش محمد نافع
بولاق النكرو - الجيزة

١٩٥٧/١٢
٢٥٧ ١٢ ٣٠ ٠١ ٠٠٤ ٠١

KZ0767025

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٢٠٢٥/٠١ ٢٥٧١٢٣٠٠١٠٠٤٠١
ربة منزل
أنثى
مسلمة
متزوجة
محمد ابراهيم سليمان الهيتي
البطاقة سارية حتى ٢٠٣٢/٠١/٢٩

